A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

Dr Shakya Peiris
Everlab

Dear Dr Peiris,

Re: Ms Alexis-Jon Keeton
DOB: 26/05/1982 Phone: 0416818499
Address: 2312/500 Elizabeth Street, MELBOURNE, VIC, 3000
Email: keeton.lexi@gmail.com

Summary

- **Normal colonoscopy with no evidence of polyps or significant pathology.**
- **Gastroscopy revealing focal intestinal metaplasia in the stomach and no active coeliac disease.**
- **Symptoms consistent with pelvic floor dyssynergia.**
- **For referral to a bowel-specific pelvic floor physiotherapist and surveillance gastroscopy in four years.**
- **No action required by you.**

I had the pleasure of reviewing Alexis-Jon following her gastroscopy and colonoscopy. You will be pleased to know that the colonoscopy was excellent, revealing a healthy-looking colon with no polyps or other pathology.

The gastroscopy excluded several conditions. Biopsies of the small intestine were normal, with no active evidence of coeliac disease. I should note that Alexis-Jon has been following a gluten-free diet for some time, which can occasionally suppress histological markers, but as she is happy to remain gluten-free, we do not need to pursue further testing or gluten challenges at this stage. We also confirmed she is negative for *Helicobacter pylori*.

In the distal stomach, I noted an area of altered appearance, and biopsies confirmed focal intestinal metaplasia. I discussed the clinical significance of this with her. Given that this is a localized area and she has no family history of gastric cancer, her absolute risk of progression is exceedingly low. While some guidelines suggest surveillance is not strictly necessary for focal metaplasia in Caucasian patients without risk factors, Alexis-Jon prefers a preventative approach.

Impression and Plan

Regarding her symptoms, she continues to experience some urgency, often associated with dietary triggers like garlic or spicy foods. However, her most bothersome symptom is a sense of incomplete evacuation, which occurs about 50% of the time. Interestingly, she noted significant improvement in her regularity after receiving colonic irrigation at a retreat, which is often a clinical clue toward an outlet obstruction issue. Combined with her history of discomfort during speculum examinations, I believe her symptoms are consistent with pelvic floor dyssynergia—specifically a tight, poorly relaxing pelvic floor.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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I have recommended she see a bowel-specific pelvic floor physiotherapist to retrain the coordination of her evacuatory mechanics. I have recommended the physiotherapists at Central Melbourne Gastroenterology (cc'ed) In terms of long-term monitoring, I have placed her on a four-year recall for a surveillance gastroscopy.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Central Melbourne Physiotherapists

Investigation Results:

KEETON, ALEXIS-JON : SVH 1112163, SVP 926853

DOB: 26/05/1982 Age: 43 years

Unit 2302, 27 Therry St, Melbourne 3000

Results from 24/04/2026 ..... Episode# 26P265145 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B010762 **CLINICAL NOTES** Gluten intolerance. Gas = antrum/pyloric =?IM. Duo = N. Random colon = N. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". Two mucosal fragments measuring 2-3 mm. A1. 2. Specimen labelled "Duodenum bx". Four mucosal fragments measuring 2-4 mm. A1. 3. Specimen labelled "Random Colon bx". Three mucosal fragments measuring 1-4 mm. A1. **MICROSCOPIC DESCRIPTION** 1. Sections show a fragment of antral type gastric mucosa including muscularis mucosa. There is focal intestinal metaplasia without dysplasia of complete type. Background mild chronic chemical gastropathy is seen. No acute inflammation, ulceration or granuloma formation is present. Helicobacter pylori micro-organisms are not identified. 2. Sections show fragments of duodenal mucosa including muscularis mucosa and Brunner's gland. The villous architecture is preserved. No intra-epithelial lymphocytosis is seen. No acute inflammation, ulceration or granuloma formation is noted. No dysplasia is present. 3. Sections show fragments of large intestinal mucosa including muscularis mucosa. The crypt architecture is preserved. No acute inflammation, ulceration or granuloma formation is seen. No thickening of the subepithelial collagen plate is noted. No intra-epithelial lymphocytosis is present. No dysplasia is identified. **PATHOLOGIST'S OPINION** **1. Gastric biopsy: - Focal intestinal metaplasia without dysplasia. - Background mild chronic chemical gastropathy, benign. 2. Duodenal biopsy: Within normal morphological limits. 3. Random colon biopsy: Within normal morphological limits.** Pathologist: Dr Zakariya Kanaan

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